



DEFENSE HEALTH AGENCY
GENERAL LEONARD WOOD COMMUNITY HOSPITAL
4430 MISSOURI AVENUE
FORT LEONARD WOOD, MO 65473

MCXP-CCS

26 MARCH 2026

MEMORANDUM OF AGREEMENT
BETWEEN
PRIMARY CARE SERVICE LINE, INPATIENT HOSPITALIST SERVICE
AND
SURGICAL SERVICE LINE, GENERAL AND ORTHOPEDIC SURGERY SERVICE

1. PURPOSE. This Memorandum of Agreement (MOA) establishes the responsibilities for inpatient admission, primary team ownership, after-hours and weekend/holiday coverage for patients admitted with primary general surgical or orthopedic surgical complaints at General Leonard Wood Community Hospital (GLWCH). This MOA supersedes all prior MOAs between these departments regarding inpatient admission and coverage responsibilities.

2. APPLICABILITY. This MOA applies to all patients admitted to GLWCH with a primary general surgical or orthopedic surgical indication for admission.

3. PRIMARY TEAM OWNERSHIP DURING DAYTIME HOURS (0600–1800, Monday-Friday).

- a. The admitting surgical service (general surgery or orthopedic surgery) will serve as the primary team of record for all patients admitted for primary general surgical or orthopedic indications.
- b. The general surgery or orthopedic team is responsible for all aspects of patient care including, but not limited to: admission orders, history and physical documentation, daily rounding with progress notes, surgical consent, transfusion consent, preoperative and postoperative orders, perioperative antibiotics and analgesia, deep venous thrombosis (DVT) chemoprophylaxis, physical therapy consultation, weight-bearing and activity orders, discharge planning, discharge summary documentation, discharge orders, and coordination with Nurse Case Management for surgical disposition and follow-up.
- c. During daytime hours, nursing staff will direct all patient care issues to the general surgical or orthopedic team.

4. AFTER-HOURS COVERAGE & ADMITTING (1800–0600, Sunday-Thursday).

- a. The general surgical or orthopedic team will provide a verbal sign-out to the night shift MOD at the end of daytime hours. The verbal sign-out will include, at a minimum: active

patient list, pertinent clinical issues, current plan of care, anticipated concerns, and specific guidance for management of foreseeable complications.

- b. The night shift MOD will serve as the provider for acute issues only during after-hours periods. The MOD does not assume primary team ownership. A transfer note and/or order is not required. The general surgery or orthopedic team remains the primary team of record. The night shift MOD will provide a verbal sign-out to each respective surgical team at the end of night shift, reviewing any acute overnight events.
- c. During after-hours periods, nursing staff will direct all patient care issues to the MOD as the covering provider. The MOD will manage acute medical issues within their scope and clinical judgment. Any acute events will be documented in the chart as a cross-cover note in SOAP format. For surgery-specific concerns or issues outside the MOD's comfort level, the MOD will contact the general or orthopedic surgeon on call by phone for guidance.
- d. The general surgeon or orthopedic team will remain reachable by phone during all after-hours periods for consultation by the MOD as needed.
- e. The MOD will admit general surgery and orthopedic patients after-hours in non-emergent/urgent cases when clinically appropriate. The general or orthopedic surgeon must be consulted by the Emergency Department (ED) provider and agree to accept the patient prior to contacting the MOD for admission. After accepting the patient, the MOD will contact the general or orthopedic surgeon by phone to review the plan of care. Any patients admitted by the MOD after-hours will be formally transferred to the general surgery or orthopedic team at the end of night shift.

5. WEEKEND AND FEDERAL HOLIDAY COVERAGE (Friday 1800 – Monday 0600)

- a. If requested by the general or orthopedic surgery team, the MOD will accept formal transfer of surgical patients on weekends and federal holidays.
- b. The general or orthopedic surgeon will provide a verbal sign-out to the MOD and place a transfer order. Surgery recommendations for ongoing patient care will be clearly documented in the patient's chart. The MOD will complete a transfer acceptance note. After accepting a patient for transfer, the MOD is responsible for all aspects of patient care, including rounding and daily progress notes.
- c. If a discharge is anticipated over the weekend, the general surgeon or orthopedic team will ensure surgical follow-up is scheduled. Specific recommendations for patient activity, discharge medications (such as DVT chemoprophylaxis or post-operative analgesia) and disposition will be documented in the patient's chart by the primary surgical service. The MOD is responsible for writing the discharge summary and placing the discharge order.

- d. The general surgeon or orthopedic team will remain reachable by phone during weekend and federal holiday coverage for consultation by the MOD as needed.

6. CONSULTATION TO PRIMARY CARE SERVICE LINE.

- a. The general surgeon or orthopedic team may consult the MOD for formal medical consultation on patients with complex medical comorbidities during daytime hours.
- b. The MOD will function as a consulting service only. The surgical or orthopedic team retains primary team ownership and responsibility for the patient. Routine medical co-management by the MOD is not authorized unless mutually agreed upon by both teams on a case-by-case basis.

7. GENERAL PROVISIONS.

- a. This MOA takes effect upon signature and expires two years from the date of signature.
- b. This MOA will be reviewed annually on or around the anniversary of its effective date.
- c. This MOA may only be modified in writing by the undersigned or successor in the position of Deputy Commander for Clinical Services.
- d. Any disputes relating to this MOA will be elevated to the Deputy Commander for Clinical Services for resolution.
- e. This MOA supersedes all previous MOAs between the Surgical Services Department, Orthopedic Surgery Department, and Primary Care Service Line regarding inpatient admission and coverage responsibilities.

7. The POC for this memorandum is the undersigned, LTC Matthew D. Holtkamp, at matthew.d.holtkamp.mil@health.mil or (515) 371-3630.

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MATTHEW D. HOLTKAMP
LTC, MC
Deputy Commander for Clinical Services